

Apollo Diagnostics & Research Centre

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DIABETIC PROFILE & COMPREHENSIVE METABOLIC PANEL

Patient Name	XXX	Patient ID	APXX-2026-XXXX	Report No.	RPT-XX-052026
Age / Gender	XX Yrs / X	Date of Birth	XX/XX/XXXX	Sample Collected	20-May-2026, 07:15 AM
Referring Doctor	Dr. XXX	Contact	+91-XXXXXXXXXX	Report Generated	20-May-2026, 04:30 PM
Address	XXXX, Bengaluru	Sample Type	Fasting Venous Blood	Fasting Duration	12 Hours

SECTION 1 — DIABETES MARKERS

Test Name	Result	Unit	Reference Range	Status
Fasting Blood Glucose (FBG)	148	mg/dL	70 – 100	HIGH
Post Prandial Glucose (2h PP)	224	mg/dL	< 140	HIGH
HbA1c (Glycated Haemoglobin)	7.8	%	< 5.7	CRITICAL
Estimated Average Glucose (eAG)	177	mg/dL	< 154	HIGH
Fasting Insulin	18.4	μIU/mL	2.6 – 24.9	NORMAL
HOMA-IR (Insulin Resistance Index)	6.8		< 2.5	HIGH
C-Peptide (Fasting)	2.1	ng/mL	0.9 – 4.0	NORMAL

SECTION 2 — LIPID PROFILE

Test Name	Result	Unit	Reference Range	Status
Total Cholesterol	214	mg/dL	< 200	ELEVATED
LDL Cholesterol (Friedewald)	138	mg/dL	< 100	HIGH
HDL Cholesterol	44	mg/dL	> 60	LOW
Triglycerides	188	mg/dL	< 150	ELEVATED
VLDL Cholesterol	37.6	mg/dL	< 30	ELEVATED
Total Cholesterol / HDL Ratio	4.9		< 4.5	ELEVATED
LDL / HDL Ratio	3.1		< 2.5	HIGH
Non-HDL Cholesterol	170	mg/dL	< 130	HIGH

SECTION 3 — RENAL FUNCTION TESTS

Test Name	Result	Unit	Reference Range	Status
Blood Urea Nitrogen (BUN)	22	mg/dL	7 – 25	NORMAL
Serum Creatinine	1.2	mg/dL	0.7 – 1.3	NORMAL
eGFR (CKD-EPI)	72	mL/min/1.73m ²	> 60	NORMAL
Uric Acid	6.8	mg/dL	3.5 – 7.2	NORMAL
Urine Microalbumin	38	mg/L	< 30	ELEVATED

Urine Creatinine	94	mg/dL	20 – 300	NORMAL
Albumin / Creatinine Ratio (ACR)	40.4	mg/g	< 30	ELEVATED

SECTION 4 — LIVER FUNCTION TESTS (LFT)

Test Name	Result	Unit	Reference Range	Status
Total Bilirubin	0.9	mg/dL	0.2 – 1.2	NORMAL
Direct Bilirubin	0.3	mg/dL	0.0 – 0.4	NORMAL
Indirect Bilirubin	0.6	mg/dL	0.1 – 0.9	NORMAL
SGOT / AST	42	U/L	< 40	ELEVATED
SGPT / ALT	56	U/L	< 40	ELEVATED
Alkaline Phosphatase (ALP)	88	U/L	44 – 147	NORMAL
Total Protein	7.2	g/dL	6.4 – 8.3	NORMAL
Serum Albumin	4.1	g/dL	3.5 – 5.0	NORMAL
Globulin	3.1	g/dL	2.0 – 3.5	NORMAL
A/G Ratio	1.3		1.1 – 2.2	NORMAL

SECTION 5 — COMPLETE BLOOD COUNT (CBC)

Test Name	Result	Unit	Reference Range	Status
Haemoglobin (Hb)	12.4	g/dL	13.5 – 17.5	LOW
Total RBC Count	4.6	mill/μL	4.5 – 5.5	NORMAL
Haematocrit (PCV)	38	%	40 – 50	LOW
MCV	82	fL	80 – 100	NORMAL
MCH	26.8	pg	27 – 33	LOW
MCHC	32.6	g/dL	31.5 – 34.5	NORMAL
Total WBC Count	9,200	cells/μL	4,500–11,000	NORMAL
Neutrophils	64	%	40 – 70	NORMAL
Lymphocytes	28	%	20 – 40	NORMAL
Platelet Count	2.1	lakh/μL	1.5 – 4.0	NORMAL

SECTION 6 — THYROID & MICRONUTRIENTS

Test Name	Result	Unit	Reference Range	Status
TSH (Thyroid Stimulating Hormone)	3.8	μIU/mL	0.4 – 4.0	NORMAL
Free T3	3.1	pg/mL	2.3 – 4.2	NORMAL
Free T4	1.1	ng/dL	0.89 – 1.76	NORMAL
Vitamin D (25-OH)	18	ng/mL	30 – 100	LOW
Vitamin B12	214	pg/mL	200 – 900	NORMAL
Serum Magnesium	1.6	mg/dL	1.7 – 2.2	LOW
hs-CRP (Cardiac Risk Marker)	4.2	mg/L	< 1.0	HIGH

CLINICAL SUMMARY & INTERPRETATION

1. **Diabetes Control (POOR):** FBG 148 mg/dL, PP Glucose 224 mg/dL and HbA1c 7.8% indicate inadequately controlled Type 2 Diabetes Mellitus. HOMA-IR of 6.8 confirms significant insulin resistance. Immediate medication review and dietary intervention recommended.
2. **Dyslipidaemia (MODERATE RISK):** Elevated LDL (138), low HDL (44), high Triglycerides (188) and elevated hs-CRP (4.2) together suggest moderate-to-high cardiovascular risk. Statin therapy and lifestyle modification are advised.
3. **Early Nephropathy:** Urine Microalbumin 38 mg/L and ACR 40.4 mg/g indicate early diabetic nephropathy. Kidney function tests should be repeated in 3 months. ACE inhibitor therapy may be considered.
4. **Mild Anaemia:** Haemoglobin 12.4 g/dL, low PCV and MCH suggest mild anaemia, possibly nutritional. Iron studies and dietary counselling recommended.
5. **Vitamin D Deficiency:** 25-OH Vitamin D at 18 ng/mL is below optimal. Supplementation (60,000 IU/week x 8 weeks) advised.

** This report is for clinical reference only and does not constitute a diagnosis. Please consult your treating physician.*

Technician: XXX Lab Tech ID: XXXX	Verified by: Dr. XXX MD Pathology Reg. No: XXXX	Authorised by: Dr. XXX Chief Pathologist
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